



— PORTFOLIO PROJECT 18 · 2026-2027

Insurance Verification Evidence & Patient
Financial Clarity Intelligence™

A student-developed healthcare operations case study that converts a basic insurance-verification checklist into an evidence-based readiness system for exact-plan identification, ASC X12 270/271 eligibility, service-specific benefits, network status, prior authorization, coordination of benefits, patient financial communication, exception ownership, synthetic UAT, and verified closure.

Created by Kori Pickle, BSHA Candidate, University of Phoenix. Educational, simulated, and no PHI. No real payer portals, claims, EHRs, employer systems, patient records, or reimbursement outcomes were used.

REVIEW SIMULATED CASE

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— HEALTHCARE
OPERATIONS
INTELLIGENCE ENGINE™

Eligibility
Benefits
Network
Authorization
Evidence & Ownership

CORE QUESTION

Where
did the
workflow
first lose
control?

Kori Pickle · BSHA
Candidate

EXECUTIVE BREAKDOWN

Insurance verification is not one check and not a guarantee of payment.

Eligibility

Confirms enrollment and coverage information for a specific date or period. CMS identifies the ASC X12N 270/271 as the HIPAA-adopted eligibility inquiry and response transaction.

Benefits

Clarifies service categories, exclusions, limits, deductibles, copays, coinsurance, and other plan details that may affect readiness and patient communication.

Authorization

Confirms whether preapproval is required and whether the approval matches the service, provider, location, units, and dates. Preauthorization is not a promise of payment.

Readiness

Requires exact-plan identity, evidence, ownership, deadlines, patient-safe communication, recheck triggers, and verified closure across all required controls.

“Verified” should be the output of completed sub-controls - not a vague checkbox.

CONTROL DISTINCTION

Separate the decisions that one status often hides.

Control	Question	What it does not prove
Exact payer and plan	Which insurer, product, line of business, payer ID, and effective dates apply?	That the service is covered or the provider is in network.
Eligibility	Is coverage active for the date of service?	That the payer will reimburse the claim.
Benefits	What service-specific coverage, exclusions, limits, and patient cost-sharing apply?	That authorization, medical-necessity, or documentation requirements are satisfied.
Network	Are the provider, facility, and ancillary entities participating for the exact product?	That every service is covered or authorized.
Prior authorization	Is approval required and does the scope match?	That payment is guaranteed.
Patient financial clarity	What is known, estimated, uncertain, or unresolved?	That the final bill will exactly match the estimate.

SIMULATED CASE

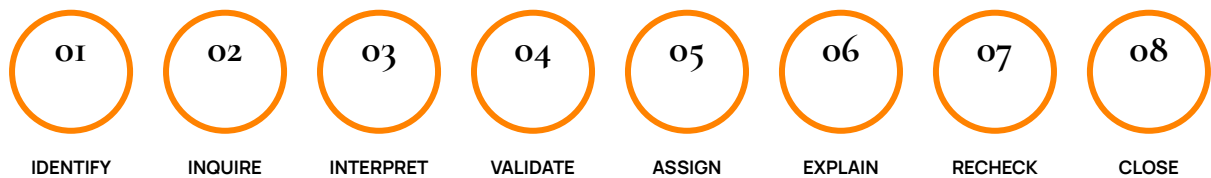
Active coverage created a false sense of readiness.

A simulated patient is scheduled for advanced outpatient imaging. Registration captures the insurance card and receives an active eligibility response. The account is marked “verified” and advances, but the exact product, imaging-specific benefits, facility network status, authorization scope, and patient financial assumptions remain incomplete.

Visible problem	The payer later denies the claim for missing or mismatched authorization information.
Operational defect	One generic status represented eligibility, benefits, network, authorization, cost, and claim readiness.
Patient-impact defect	The patient heard “your insurance is verified” and reasonably interpreted that statement as a promise of coverage.
First loss of control	“Active” was treated as “ready” before the required sub-controls had evidence and ownership.
Control response	Separate statuses, scope validation, exception queues, patient-safe scripts, recheck triggers, and verified closure.

— READINESS CHAIN

Eight stages from insurance identification to verified closure.



A note records a problem. A queue creates ownership, timing, visibility, and escalation.

CONTROL REDESIGN

Insurance Verification Evidence Control Matrix™

Control	Required evidence	Status design	Escalation trigger
Identity and plan	Member ID, subscriber, payer, exact product, payer ID, dates, source, timestamp	Confirmed / conflict / unresolved	Card, registration, and payer response disagree
Eligibility	Date-specific status and response details	Active / inactive / future / limited / unavailable	Service approaching without reliable response
Benefits	Service type, limits, exclusions, accumulators, response notes	Covered / limited / excluded / unclear	Service-specific answer missing or contradictory
Network	Exact plan, provider, facility, ancillary entity, location, source	In / out / unknown / exception	Any component remains unknown before service
Authorization	Requirement, number, service, units, provider, location, dates, documents	Not required / pending / approved / denied / expired	Scope mismatch, expiration, or missed deadline
COB / other coverage	Other payer, primary-secondary decision, source, date	Resolved / investigation / changed	Multiple coverage or conflicting information
Patient communication	Known cost-sharing, estimate assumptions, uncertainty, next steps	Explained / pending / interpreter needed	Patient believes verification guarantees payment

Control	Required evidence	Status design	Escalation trigger
Closure	Completed sub-controls, exception approval, owner, timestamps, release	Ready / conditional / hold / canceled	Any required status lacks evidence

— SYNTHETIC UAT

Test the exceptions before a real patient or claim absorbs the defect.

Wrong product Ready status is blocked until the exact product and payer ID are confirmed.	Incomplete response Eligibility can be active while service-specific benefits remain unresolved.	Scope mismatch Approval for the wrong provider, date, location, service, or units does not satisfy readiness.
Network gap Overall readiness remains open when facility or ancillary network status is unknown.	Coverage change Plan changes reset dependent statuses while preserving the previous audit trail.	Conflicting sources Contradictory electronic and phone responses create an owned exception, not a silent guess.

CAREER VALUE

Recruiter-visible proof for 2026-2027 roles.

Patient Access

Exact-plan identification, eligibility, network awareness, patient communication, and appointment readiness.

Prior Authorization

Requirement identification, scope matching, evidence, aging, ownership, and escalation.

Revenue-Cycle Support

Front-end denial prevention, COB awareness, routing readiness, and closed-loop root-cause feedback.

Health Informatics

Status design, provenance, audit trails, exception queues, business rules, and data quality.

Implementation Support

Workflow requirements, synthetic UAT, acceptance criteria, defect tracking, and adoption safeguards.

Patient-to-Professional

Clear language, estimate limitations, visible uncertainty, and protection from unsupported promises.

I separate eligibility from complete service readiness and design the evidence, ownership, testing, and communication controls that prevent false-ready work.

— OFFICIAL SOURCES

Current federal and consumer-facing reference points.

[CMS: Health Plan Eligibility Benefit Inquiry and Response](#)

[CMS: Operating Rules FAQs](#)

[HealthCare.gov: Deductible](#)

[HealthCare.gov: Copayment](#)

[HealthCare.gov: Coinsurance](#)

[HealthCare.gov: Out-of-pocket maximum](#)

[HealthCare.gov: Preauthorization](#)

[CMS: Interoperability and Prior Authorization Final Rule CMS-0057-F](#)

Student-developed, simulated, educational, and no PHI. This work does not represent employer operations, payer determinations, coding authority, compliance authority, clinical judgment, or guaranteed financial outcomes.